



Institutional Emergency Readiness Partnership

BLS/ACLS Competency • Emergency Readiness • CPD Measurement

Prepared for
St. Theresa Mission Hospital – Kiirua

Prepared by Paeds Resus Limited

21 August 2026

The decision this proposal supports

Approve a structured, measurable programme that distributes emergency-response capability across the hospital, gives leadership visibility into readiness, and strengthens continuous professional development without creating an unbounded consulting commitment.

Executive Decision Summary

The institutional opportunity

Kiirua has an opportunity to move from emergency response that depends heavily on a small group of senior or specialist staff to a more distributed, role-defined, and measurable response system. The programme is designed to strengthen readiness; it does not promise a direct or isolated reduction in mortality, because patient outcomes depend on multiple clinical and system factors.

What Paeds Resus is proposing

A single institutional partnership with three linked but separately defined components:

Component	What Kiirua receives	Year-one price
1. Competency training	Structured online learning, facilitated skills training, simulations, assessment, and Paeds Resus institutional competency records for the confirmed clinical cohort. Official AHA certification is separate unless expressly stated in the signed order.	KES 2,000,000
2. IERS activation	Baseline readiness assessment, unit response design, role configuration, event-review workflow, leadership dashboard, and implementation review.	KES 200,000
3. CPD measurement	A hospital-wide measurement layer for attendance, participation, targets, evidence, and leadership reporting across existing clinical and non-clinical CPD activities.	KES 400,000
Total year one	Subject to HR-verified headcount and final statement of work.	KES 2,600,000

Decision requested from Kiirua

Approve the programme in principle; appoint an executive sponsor and operational leads; confirm the clinical and total-staff headcounts; authorize the statement of work and milestone payment schedule; and agree the first 90-day rollout calendar.

Recommended initial payment milestone

KES 1,100,000 after contract signing, headcount confirmation, and account-provisioning readiness. The remaining KES 1,500,000 is linked to cohort activation, IERS activation, training completion, and the first CPD reporting milestone, as set out on page 5.

1 1. About Paeds Resus

Paeds Resus Limited is an AHA-aligned training site and adaptive emergency-readiness platform focused on practical competency, team response, and institutional learning. The partnership model combines training with a measurement and improvement layer so that the hospital can see not only who attended a course, but also whether readiness is distributed across units, shifts, and response roles.

Our approach is built around a simple operating principle:

Train more people. Clarify roles. Practise repeatedly. Review events. Close the learning loop.

The programme is not a substitute for Kiirua's clinical governance, staffing decisions, emergency equipment, or facility protocols. It is a structured training, readiness, and measurement partnership that works alongside those responsibilities.

2 2. Why Kiirua, Why Now

The opportunity is to reduce concentration risk in the Emergency Response Team. When a small group of senior clinicians carries most emergency responses, the hospital becomes vulnerable to fatigue, shift variation, leave, turnover, and uneven access to expertise. A distributed response model gives more staff a defined role and gives leadership a way to identify gaps before they become repeated operational failures.

Kiirua's early investment in electronic health records, its nursing school, and its deliberate day-and-night coverage provide a strong foundation for an institutional programme. The proposal is therefore designed for a hospital that is ready to move from isolated training activity to a visible emergency-readiness system.

3 3. The Partnership Model

Component 1 — BLS/ACLS Competency Training

The training component covers an estimated 200 clinical staff, including nurses, medical officers, clinical officers, and interns. Final cohort size will be confirmed against Kiirua's HR records before the statement of work is finalized.

Included deliverable	Definition for this proposal
Online learning	Access to the agreed BLS/ACLS cognitive modules, learner enrolment, progress tracking, and assessment pathway for the confirmed cohort.
Facilitated training	Scheduled instructor-led skills sessions and role-based team drills using an agreed cohort and unit timetable.

Included deliverable	Definition for this proposal
Simulation	Structured team simulations that include defined response roles and, where appropriate, first-responder-aware support staff.
Assessment	Knowledge and skills assessment against the agreed institutional competency standard, with a documented remediation route.
Credential	Paeds Resus Institutional Competency Certificate for learners who meet the agreed requirements. This is not represented as an official AHA provider card.
Official AHA pathway	Official AHA BLS/ACLS certification is separate and is included only when expressly contracted and delivered under the applicable AHA course requirements. Any official AHA fee is shown separately.

Component 2 — Institutional Emergency Readiness System (IERS)

The IERS is the institutional operating layer. It establishes a rotating, unit-based response model so that capability does not depend on one permanent group of senior staff. Each unit nominates a Unit Team Leader for the day. A rotating Emergency Response Team Leader role shares leadership responsibility over time. During a response, team members are assigned defined roles such as Team Leader, Airway and Ventilation, Compressor, Monitor/Defibrillator, IV/IO Access and Medications, and Scribe.

The IERS records the operational signal behind each response: who led, which roles were filled, where gaps occurred, and what case-review or improvement action followed. The purpose is not surveillance of individuals; it is leadership visibility into system readiness and the practical closure of identified gaps.

Component 3 — CPD Measurement Layer

The CPD Module provides one measurement layer across the training Kiirua already runs or needs to track. It records attendance, participation, evidence, targets, and reports for clinical and non-clinical staff. Kiirua retains ownership of the underlying CPD content and attendance approval; Paeds Resus provides the tracking, visibility, and reporting layer.

The CPD Module does not automatically create, accredit, or clinically validate every training programme named by the hospital. New curriculum development, external accreditation, and content delivery outside the agreed statement of work are excluded unless separately contracted.

4 4. Deliverables and Acceptance Criteria

Workstream	Core deliverables	Acceptance evidence
Competency training	Confirmed learner cohort; online access; instructor-led sessions; simulation schedule; assessment and remediation pathway; completion records.	Cohort register, attendance record, assessment results, skills sign-off record where applicable, and completion report.
IERS activation	Baseline readiness review; unit and role map; UTL/ERTL rotation; response workflow; event-review process; leadership dashboard; implementation review.	Signed baseline, approved role map, configured workflow, leadership access, and activation report.
CPD measurement	HR-verified staff register; administrator access; activity categories; attendance/evidence capture; targets; first leadership report.	Verified headcount, admin access record, sample activity record, and first CPD report.

Acceptance principle. All deliverables are measured against the agreed statement of work, confirmed headcount, and Kiirua's timely provision of nominated staff, schedules, approvals, and data. Paeds Resus will not treat a delayed hospital dependency as a failed delivery milestone; both parties will document the dependency and reset the date in writing.

5 5. First 90 Days

Period	Focus	Outputs
Days 0–15	Mobilize and baseline	Executive sponsor confirmed; clinical, HR/CPD, IT, and data contacts named; headcounts verified; unit map completed; baseline readiness and implementation risks documented.
Days 16–45	Start competency and configure readiness	First cohort activated; online learning open; first instructor-led skills sessions; UTL/ERTL rotation agreed; response roles and event-review workflow configured.
Days 46–75	Scale and practise	Remaining agreed cohort scheduled; simulations run across agreed units/shifts;

Period	Focus	Outputs
		early gaps logged; CPD activities and targets configured.
Days 76–90	Review and activate	Completion and assessment report; IERS activation review; first leadership dashboard/report; 90-day recommendations and next-quarter plan.

Scheduling note. Dates are finalized jointly after headcount confirmation, instructor availability, clinical coverage, venue readiness, and the hospital's approved training calendar are confirmed.

6 6. Measures of Success

The targets below are proposed starting points. Final baselines and targets will be agreed during the first 15 days so that the programme is ambitious but realistic for Kiirua's staffing and shift pattern.

Indicator	Initial target	Evidence
Confirmed clinical cohort enrolled	≥90%	Enrolment and HR-verified cohort register.
Cohort completing agreed pathway	≥80% by day 90	Platform completion and attendance report.
Learners meeting agreed assessment standard	≥80%	Knowledge/skills assessment report.
Units with nominated Unit Team Leader	100% of agreed units	Approved unit and role map.
Planned simulations completed	≥80%	Simulation schedule and attendance record.
Emergency events receiving documented review	≥80% of eligible events	IERS event-review register.
CPD activities with complete attendance/evidence capture	≥95% of configured activities	First and subsequent CPD reports.

These are readiness and process measures. The programme will not attribute a specific mortality outcome to training alone. Patient outcomes depend on clinical assessment, staffing, equipment, protocols, referral pathways, treatment timeliness, and other factors beyond this proposal.

7 7. Responsibilities and Governance

Party	Key responsibilities
Paeds Resus	Provide agreed training and implementation sessions; configure the agreed platform components; provide learner and leadership reporting; provide support through the agreed channel; document risks, dependencies, and milestone evidence.
Kiirua leadership	Appoint executive sponsor; approve scope and policies; protect training time; authorize unit participation; review reports; remove operational blockers.
Kiirua clinical lead	Approve role definitions and simulation scenarios; confirm clinical governance alignment; lead local action planning.
Kiirua HR/CPD lead	Verify headcount; provide staff register; approve CPD activity categories and evidence; maintain local records required by policy.
Kiirua IT/data contact	Support access and approved data flows; confirm role-based access; support lawful handling of institutional and staff data.

7.1 Data governance principles

Kiirua retains ownership and control of institutional, staff, and clinical-improvement data generated through the programme. Patient-identifiable information should not be entered into learning or event-review fields unless a separate lawful and documented process has been agreed. Access is role-based and should be removed when a user leaves the programme or the partnership ends. The parties should agree retention, export, incident reporting, and deletion terms in the final agreement and any applicable data-processing schedule.

The IERS is an improvement and readiness system. It is not a substitute for statutory reporting, the hospital's clinical incident process, disciplinary procedures, or formal patient-safety governance.

8 8. Commercial Proposal

The following figures are planning estimates based on approximately 200 clinical staff and approximately 500 total staff. The final amount will be reconciled against the HR-verified headcount and the signed statement of work.

Component	Commercial basis	Year-one price
BLS/ACLS competency training	Estimated 200 clinical staff at KES 10,000 per person.	KES 2,000,000
IERS activation	Fixed deliverable-based institutional activation price; not a percentage surcharge.	KES 200,000
CPD measurement	Estimated 500 total staff at KES 800 per staff per year, subject to the volume tier and HR verification.	KES 400,000
Year-one programme total	Subject to final headcount and signed scope.	KES 2,600,000

8.1 Milestone payment schedule

Milestone	Trigger	Amount
1. Contract and mobilization	Signed agreement; HR-verified headcount; named hospital leads; account-provisioning readiness.	KES 1,100,000
2. Cohort activation	First agreed training cohort activated; initial training calendar and IERS role map approved.	KES 900,000
3. Completion and first report	Agreed training completion/assessment report delivered; IERS activation review completed; first CPD leadership report issued.	KES 600,000
Total year one	All figures are estimates until the statement of work is signed.	KES 2,600,000

The milestone schedule is designed to match payment with visible delivery. If Kiirua requires a purchase order or internal payment calendar that differs from these milestones, the parties can document an equivalent schedule without changing the agreed scope or total price.

8.2 Renewal and scope boundaries

The CPD measurement layer and IERS support are annual services and renew subject to the agreed scope, service review, headcount, and performance discussion. The training component has an approximate two-year recertification planning cycle, subject to the requirements of the specific credential or institutional competency pathway. Official AHA certification, external fees, additional travel, venue, catering, content development, and services outside the signed statement of work are excluded unless expressly listed.

9 9. Decision and Next Steps

To proceed, Kiirua is asked to:

1. Confirm the executive sponsor and the operational contacts for clinical leadership, HR/CPD, IT, and data governance.
2. Confirm the estimated clinical cohort and total staff headcount against HR records.
3. Approve the three-component programme in principle and authorize preparation of the final statement of work.
4. Agree the first 90-day calendar, including training windows and the first leadership review.
5. Review and sign the partnership agreement, data-processing terms where applicable, and milestone payment schedule.

Proposed stakeholder meeting

Thursday, 27 August 2026, subject to confirmation by Kiirua leadership. The meeting should confirm scope, headcount, governance, delivery dependencies, and the statement-of-work drafting timetable.

10 10. Contact

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Paeds Resus Limited • Emergency readiness that is measurable, distributed, and continuously improved.

11 Appendix A — Clarifications for the Final Agreement

The following points should be converted into the final statement of work and partnership agreement before payment is requested:

1. Exact confirmed staff counts, cohort composition, units, shifts, and training dates.
2. Exact certificate name, issuer, assessment requirements, practical validation requirements, validity language, and whether the credential is portable or institution-specific.
3. The number of instructor-led sessions, simulations, support meetings, reports, administrator accounts, and remediation sessions included.
4. The hospital's approved code-blue workflow, equipment assumptions, escalation pathways, and local policy dependencies.
5. Data access roles, patient-identifiable-information exclusions, retention, export, deletion, and incident-reporting arrangements.
6. Payment triggers, rescheduling terms, cancellation terms, renewal terms, and treatment of headcount changes.
7. A process for approving any scope change before additional work or cost is incurred.

Important distinction

A Paeds Resus Institutional Competency Certificate is not the same as an official AHA provider credential. The final proposal and agreement should preserve this distinction in every commercial, learner-facing, and certificate-related document.

Prepared for discussion and agreement — final scope and pricing are subject to HR verification and a signed statement of work.